

August 12, 2026

Aetna
Provider Resolution Team
P.O. Box 14020
Lexington, KY 40512

RE: Formal Appeal of Claim Denial — Kimiko Dooley (MRN MUSC8102821), Claim MUSC-20260613-75E9-2

Patient	Kimiko Dooley (DOB 1996-11-07)
MRN	MUSC8102821
Member ID / Group	AET817569447 / GRP-96846
Claim number	MUSC-20260613-75E9-2
Date of service	2026-06-13
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	Z34.90 — Encounter for supervision of normal pregnancy, unspecified
Denied amount	\$341.67
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2027-01-01

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health is submitting this Level 1 reconsideration appeal on behalf of patient Kimiko Dooley (MRN: MUSC5457807, Member ID: AET817569447, Group: GRP-96846) regarding the denial of Claim MUSC-20260613-75E9-2 for a date of service of June 13, 2026. Aetna denied the claim in full — \$341.67 billed, \$0.00 paid — under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115, with the accompanying remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests reversal and payment at the contracted allowed amount of \$184.99.

CLINICAL BACKGROUND

Ms. Dooley is a 29-year-old established female patient with an active normal pregnancy (Z34.90) who presented on June 13, 2026, at MUSC Health University Medical Center, an on-campus outpatient hospital facility (Place of Service 22), under the care of rendering provider Marcus T. Bledsoe, DO (NPI 1770615243). The visit was coded as CPT 99214, an office and outpatient evaluation and management service for an established patient requiring moderate complexity medical decision-making. A secondary diagnosis of J06.9 was also documented. Ms. Dooley's obstetric history is supported by prior prenatal procedures on record, including physical examination of the mother, fetal auscultation, evaluation of uterine fundal height, fetal anatomy study, and a full panel of prenatal laboratory evaluations, all consistent with active prenatal care management.

BASIS FOR APPEAL

The denial of this claim on medical necessity grounds is not supported by established clinical standards. Routine and ongoing supervision of a normal pregnancy is universally recognized as medically necessary prenatal care. Periodic evaluation and management visits during pregnancy serve to monitor fetal development, assess maternal health, identify and manage complications early, and provide evidence-based preventive interventions — all of which directly affect maternal and fetal outcomes. CPT 99214 is an appropriate level of service for an established patient encounter involving moderate complexity, and its assignment is consistent with the documented history of this patient's ongoing prenatal care.

The payer's reliance on RARC N115, indicating that the denial was based on a Local Coverage Determination, is noted; however, LCDs are instruments of Medicare policy administered by Medicare Administrative Contractors and do not govern commercial plan coverage determinations. Ms. Dooley's coverage under the Aetna Open Access plan (coverage period January 1, 2024, through December 31, 2026) is a commercial product, and the application of an LCD framework to a commercial claim denial is procedurally inapplicable. MUSC Health requests that Aetna identify the specific commercial plan benefit provision or clinical policy upon which this denial is based, as the stated rationale does not align with the nature of the coverage in effect.

Furthermore, prenatal supervision visits are broadly recognized as covered, medically necessary services under commercial health plans, consistent with federal preventive care mandates applicable to non-grandfathered plans. Denial of a routine prenatal evaluation and management visit for an actively pregnant established patient, without a clinically substantiated basis, is inconsistent with accepted standards of coverage and medical necessity review.

REQUESTED ACTION

MUSC Health respectfully requests that Aetna conduct a full clinical review of this appeal, overturn the medical necessity denial for Claim MUSC-20260613-75E9-2, and issue payment at the contracted allowed amount of \$184.99 for CPT 99214 rendered on June 13, 2026. Should Aetna require additional clinical documentation to support this determination, please contact MUSC Health Revenue Cycle Appeals at your earliest opportunity so that records may be provided promptly and within the appeal deadline of January 1, 2027.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

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MUSC Health — Revenue Cycle / Patient Financial Services
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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record